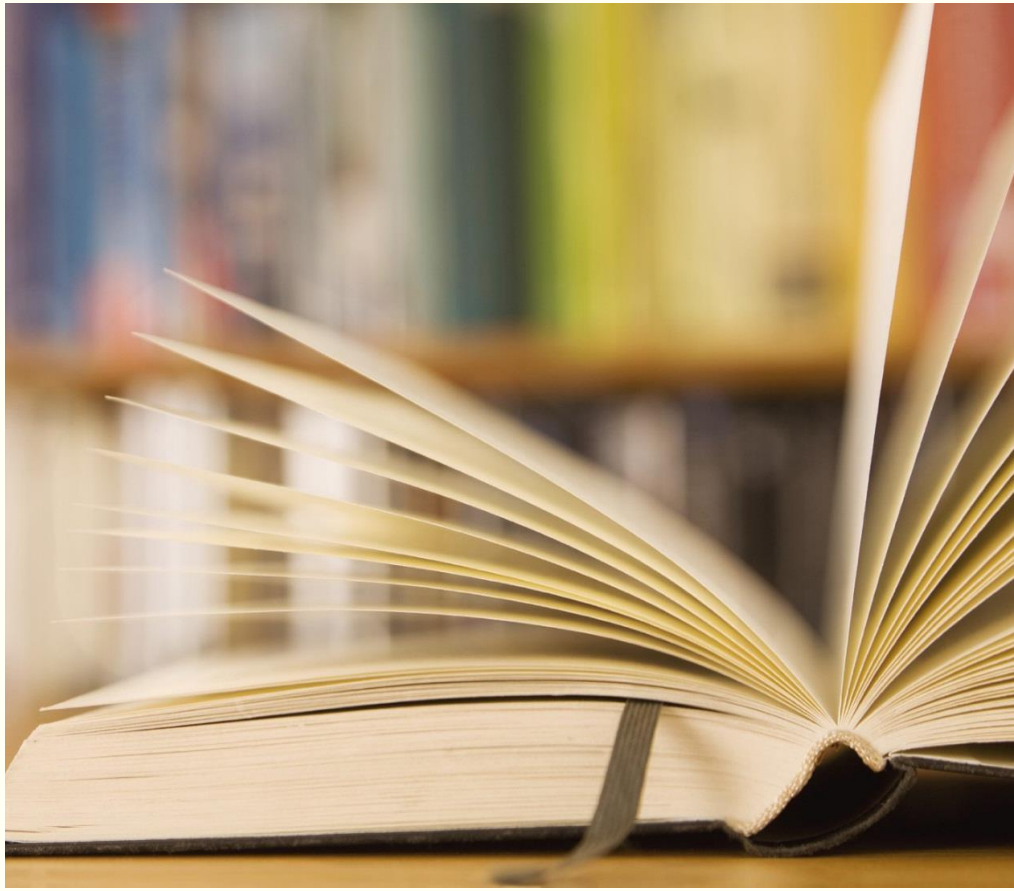
PSORIASIS

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PSORIASIS

Definition:

It is a common , chronic, recurrent, inflammatory disease of the skin characterized by: well-defined, red, scaly lesions of various sizes, covered with silvery white scales.

Aetiology:

Provocative Factors:

- Genetic predisposition.
- Immunological abnormalities.
- Trauma.
- Infection.
- Weather.
- Stress.
- Drugs: antimalarials, B-adrenergic blockers

Clinical types :

- Psoriasis vulgaris. (80-90%)
- Guttate Psoriasis.
- Discoid (Coin-shaped)
- Annular (Ring shaped)
- Scalp psoriasis.
- Nail psoriasis
- Flexural psoriasis.

- Pustular Ps. (severe)
- Erythrodermic Ps. (severe)
- Arthropathic Ps. (severe)

Clinical picture (classical) (vulgaris)

- Lesion: erythematous papules and plaques covered by silvery white scales.
- Usually bilateral and symmetrical.
- Site: any site can be affected.

(commonly extensor surfaces of the limbs, elbows, knees)



Psoriasis vulgaris





Guttate Psoriasis





Discoid Psoriasis



Annular Psoriasis

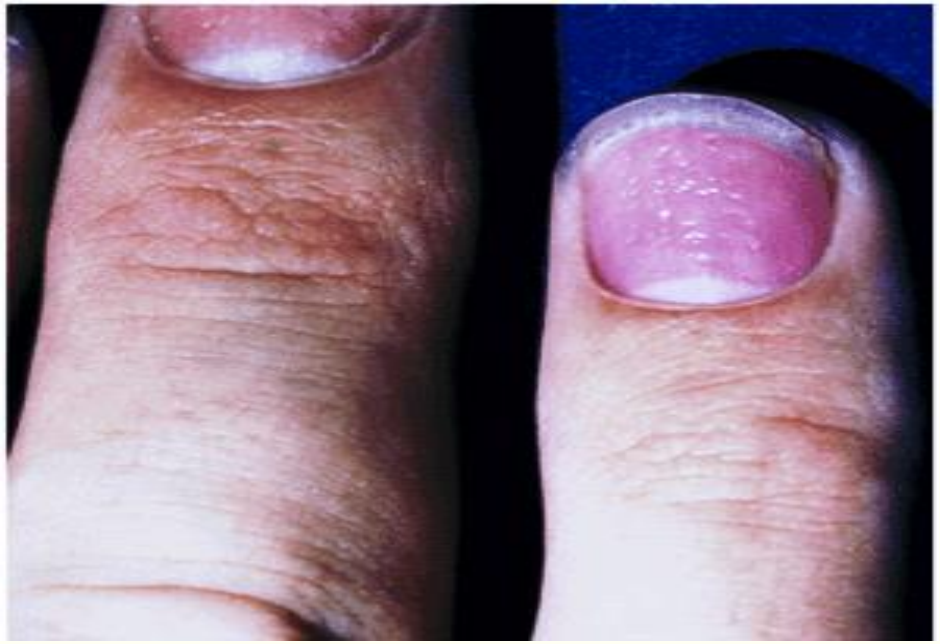


Flexural Psoriasis



Psoriasis of the Scalp







Erythrodermic Psoriasis



Arthropathic Psoriasis



Pustular Psoriasis



PSORIASIS

- **Auspitz sign:**

scraping→scales →bleeding points(dilated capillaries).

- **Koebner's phenomenon:**

It is the appearance of typical lesions at sites of injuries.

PSORIASIS

Management :

- **Reassurance and Explanation :**

Psoriasis is a chronic disease that cause psychological

stresses

1- It is not infectious
2- It is of benign nature
3- Spontaneous remission can occur
4- Therapy is available

Treatment:

A-Topical therapy

- Corticosteroids (most frequent).
- Crude coal tar 2-5%.
- Dithranol.
- Salicylic acid 3-5%.
- Vitamin D3 analogue, inhibit keratinocyte proliferation (Calcipotriol).
- Ultraviolet light: antimitotic effect

B- Intralesional Steroids

Treatment:

C- Systemic therapy

Only in severe forms of psoriasis

(erythrodermic, arthropathic, pustular)

- Methotrexate.
- Retinoids.
- Cyclosporine A.
- PUVA. It involves the use of a psoralen (P) followed by exposure to long wave ultraviolet light (UVA).
- Biologic therapy
- Corticosteroids (it is contraindicated in psoriasis vulgaris).



Any questions?

